The National Focal Point survey as evidence for the RespiCast review

A reading of the 2024 NFP survey against the review's least-cost test — what it does and does not license.

Provenance and scope (read first). *Instrument:* the 2024 ECDC survey of National Focal Points (NFPs) for respiratory viruses, run by the ECDC Respiratory Viruses & Legionella group and the Modelling team to learn **whether and how countries use mathematical modelling** to interpret surveillance data and inform decisions, and **which outputs they would prefer from ECDC modelling tools**; results were presented to the network in **October 2024**. It is a *general* modelling survey — not a RespiCast- or RespiCompass-specific evaluation — so the output-preference question (§2a) is in scope, but the instrument reports **stated awareness and preference, not measured use**. *Vintage:* fielded in **2024**, when RespiCast was ~1 year old; it informs a late-2026 decision, so its figures are ~2 years old. *Population:* **19 of ~30 NFPs** replied — national surveillance / public-health **consumers**, giving one collective **institutional position** per country (the respondent need never have opened a forecast). They are **not** the contributor modelling network (~9 teams / 28 models) and **not** the website-user survey (results due November); those three must not be conflated. *Non-response:* 19 of ~30 is not random — the absent ~11 plausibly skew **less** aware/engaged, so the figures here are, if anything, **optimistic**; every share moves ~5 points per country and subgroup counts below are single digits. *The governing question:* the review asks not only what value RespiCast *has* and *could have*, but whether that value could be had **more cheaply**. A benefit counts toward continuation only where the survey gives reason to think a cheaper route would not deliver it.

1. What the survey licenses — and the gap where the cost sits. The strongest finding is a **need**: **13 of 19 institutes (68%) have no in-house mathematical modeller**, and only one has a team above five (Figure 1). That is clear evidence ECDC should provide **some** central forecast. It is **silent** on the question that carries the cost — whether that forecast must be a ~28-model community ensemble on a hubverse platform rather than a single centrally-run model. The survey cannot compare the two, and that silence is itself decision-relevant. It sharpens against the stated use: the activity NFPs most expect modelling to inform is **situational awareness** (surveillance, **58%** "likely"), while the demanding forecasting use case — **healthcare-capacity planning** — is the one they expect **least** (**16%** "likely"; 42% "unlikely"). An undemanding use case is one a much simpler product might serve adequately. What a multi-model ensemble buys over a simple central forecast, for situational awareness, the survey cannot say — and should not be read to.

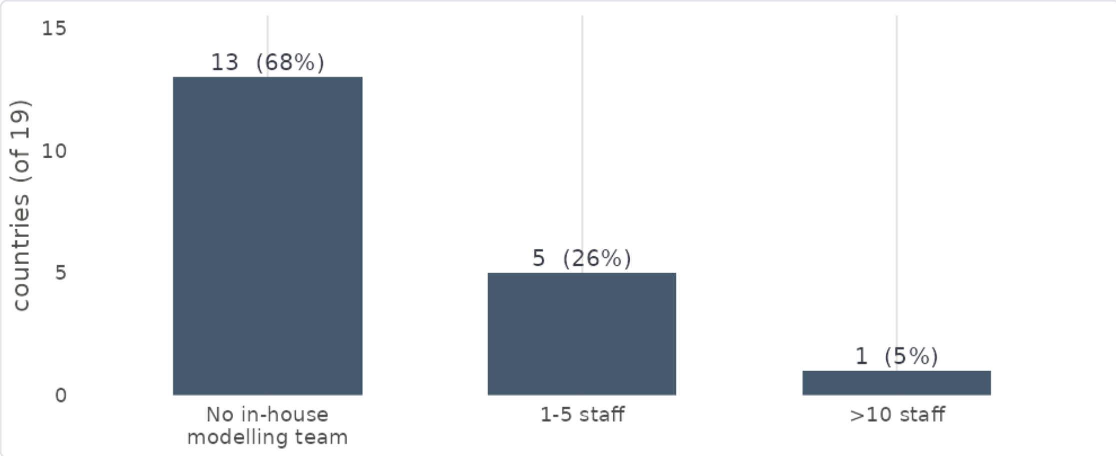

Figure 1. In-house mathematical-modelling staff at the national institute (2024 NFP survey, 19 countries). Source: `code/05_figures/fig_respicast_decision.R`.

2. Awareness — not use — and it is thinnest where the need is greatest. Q5 asked *"how aware are you of respiratory virus burden modelling work done at ECDC?"*, rated 0–5 on a slider per output. It measures **awareness, not use** — a distinction the first draft of this note blurred and this version corrects. Awareness of RespiCast averages **2.1 / 5** (18 countries) and is **not** higher where in-house capacity is absent: no-modeller countries average **2.08** (n = 13), those with a team **2.2** (n = 5). The review's discriminator — do the RespiCast-**unaware** cluster among the no-modeller countries? — lands cleanly, at small n: **all three institutes entirely unaware of RespiCast have no modelling team; every country with any in-house capacity was aware.** So the awareness gap sits exactly where the need is greatest — a **distribution / communications gap**, cheap to close, favouring *continuation-evolved*. What the survey **cannot** say is whether closing it converts into use: Q5 is awareness only, and the sole evidence on actual use is the open text (§5), where several institutes state they have **not yet used** the outputs. Awareness is a necessary, not a sufficient, condition for value.

2a. The preference question (in scope, this time). The survey explicitly asked which outputs NFPs would prefer, so the forecasts-vs-scenarios comparison is a legitimate finding, not a by-product: **15 of 19 (79%)** expect RespiCast to be useful, and among those who chose a single output, forecasts were preferred to scenarios **5:1**. In least-cost terms this supports ECDC providing **a** central forecast in preference to a scenario product — it says nothing about whether that forecast must be the multi-model hub.

3. The low score is largely recency. Awareness tracks how long an output has been in circulation: **guidance documents (since 2020) sit at 3.9**, the COVID-era hubs at ~2.7, **RespiCast (2023) at 2.1** and **RespiCompass (2024) at 1.35** — newest, least known. So RespiCast's low awareness is substantially a recency and communication-reach effect, not a signal about its value or use; closing the gap (cheap comms) would move it toward the level of the older, better-known outputs. Even this awareness reading is ~2 years stale.

4. The realised-value ceiling: a route into decisions. Independent of the product, only **9 of 19 institutes (48%)** report a clear mechanism to integrate modelling into public-health decisions; **6 (32%)** disagree (Figure 2). This caps what any forecast can achieve nationally. Whether ECDC helping to close it is cheap or a cost carried elsewhere is a cost question (§6), not free upside.

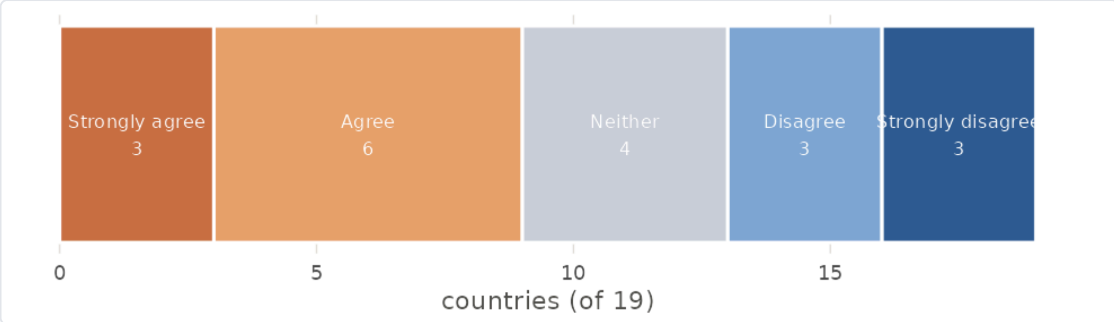

Figure 2. "There is a clear, consistent mechanism to integrate modelling outputs into public-health decisions at my institute" (2024 NFP survey, 19 countries). Source: `code/05_figures/fig_respicast_decision.R`.

5. The beneficiaries' own ask is a rival for the budget. The clearest single open comment is not a request to improve RespiCast but to fund national capacity: *"An ECDC recommendation for MSs to have adequately resourced and funded national-level modelling capacity … A sustainable European funding mechanism … Supranational modelling is useful, but national-level capacity is required to respond to the needs of each MS."* In least-cost terms this is a **challenger to the hub for the same money**, and it is the beneficiaries' stated preference. If 68%-no-modeller is the strongest survey argument *for* a central forecast, this is the strongest survey argument that the same budget might buy more capability by **funding national capacity**. Both come from this survey; both belong before the decision.

6. The improvement levers are cost lines, not free upside. The changes the survey points to are "continuation-evolved" levers, each with an owner and non-trivial effort — costs the least-cost test must weigh, not benefits:

Lever the survey points to	Owner	Order-of-magnitude effort / where costed
Plain-language, interpretable outputs for non-modellers	ECDC (hub / comms)	Low–med; recurring editorial
Short training for NFP consumers	ECDC + NFPs	Med; recurring
Published skill / accuracy track-record	ECDC + contributor teams	Med; needs an evaluation pipeline — not a survey deliverable
A mechanism to use forecasts in national decisions	Member states	Med–high; largely a national process cost
National modelling capacity + funding	Member states / EU	High; budget-scale — a strategic alternative (§5), costed elsewhere

7. What this evidence does not speak to. Actual use / uptake (Q5 is *awareness*; only the open text touches use); internal ECDC value (the ERVISS-integration interviews); surge / emergency-response capacity; forecast **accuracy or skill** (this is stated awareness, need and preference, not performance); the **contributor community** (~9 teams / 28 models — a different population); the hub-versus-simple-forecast cost question (§1); and anything **after 2024**.

8. Mapping onto the review's options (considerations, not a recommendation).

- **Continuation, as-is** — supported only for *a* central forecast, not specifically for the multi-model hub; awareness modest and, given recency, likely to rise only toward the ~2.7 level of the older outputs.

- **Continuation, evolved** — the cheapest well-evidenced lever is closing the awareness/distribution gap among no-modeller countries; interpretability and a decision pathway follow, at cost (§6).
- **Descoping (core / modularise / caretaker / transfer)** — consistent with the undemanding stated use case (situational awareness; capacity-planning only 16%): a lighter product might serve it. The survey neither rules this in nor out — it does not compare the hub against a simpler forecast.
- **Termination** — not supported: 68%-no-modeller is a clear need for *some* central forecast; but "some forecast" is not "this hub".
- **Rival use of funds** — the beneficiaries' own funding ask (§5) is a live alternative for the same budget.

All figures and numbers are generated from the de-identified survey by `code/05_figures/fig_respicast_decision.R`*. Subgroup counts are single digits; percentages are indicative (±~5 points per country) and, given non-response, likely optimistic. The open comments add that forecasts are seen as more reliable and actionable than long-horizon scenarios — consistent with the §2a preference — but remain qualitative signal, not measured use.*